

SRA-001 Module 2: History of Cupping Around the World

Instructor Guide

CONFIDENTIAL — SRA Faculty Only

Module at a Glance

Attribute	Detail
Module	M2 —History of Cupping Around the World
Duration	8 hours (2 lessons x 4 hours)
Lessons	2.1: Ancient Civilisations / 2.2: Islamic Golden Age to Today
Slides	15 slides
Learning Domains	K1, I1
Prerequisites	Module 1: Introduction to Hijama
Equipment	Projector, whiteboard, timeline handout

Learning Outcomes

Upon completion, students will be able to:

1. Trace the documented history of cupping from ancient Egypt to the present day
2. Describe the contributions of Egyptian, Chinese, and Greek civilisations
3. Analyse the contributions of key Islamic scholars to Hijama literature
4. Explain the methodology of Ibn al-Qayyim and the significance of Zad al-Ma’ad
5. Evaluate the impact of colonialism and the modern revival
6. Apply source criticism to historical claims

Lesson 2.1 — Ancient Civilisations (4 hours)

Timing Breakdown

Segment	Duration	Activity
Opening	10 min	Recap Module 1, niyyah, lesson overview
Ancient Egypt	45 min	Slide 4: Ebers Papyrus, metu theory, key contributions
Ancient China	45 min	Slide 5: TCM integration, instrument evolution
Break	15 min	

Table 2 – continued

Segment	Duration	Activity
Ancient Greece	45 min	Slide 6: Hippocratic Corpus, four humours
Historical Timeline	30 min	Slide 7: Comparative overview, source criticism
Knowledge Check	15 min	Quiz and discussion
Practical Activity	30 min	Timeline construction exercise
Reflection	10 min	“Standing on the shoulders of giants” prompt
Summary	5 min	Key takeaways, preview Lesson 2.2

Teaching Notes by Slide

Slide 4: Ancient Egypt

- Show the Ebers Papyrus image — emphasise this is a real document, not legend
- Discuss the concept of metu — channels of the body — as a precursor to other channel theories
- Scholarly note: Distinguish between documented claims (c. 1550 BCE) and speculation
- Ask: “Why does it matter that cupping has a 3,500-year history?”

Slide 5: Ancient China

- Emphasise the systematic nature of TCM cupping — it wasn’t just folk practice
- Instrument evolution: show how materials changed with available technology
- TCM theory (meridians, qi, blood stagnation) differs from Hijama’s Islamic framework — both are valid traditional systems
- Clinical pearl: TCM research informs Hijama practice despite different theoretical frameworks

Slide 6: Ancient Greece

- The four humours theory is historically important but scientifically outdated — be clear about this
- The key Greek contribution was the SYSTEMATIC approach: diagnosis → indication → contraindication → technique → follow-up
- This five-step process mirrors the SAFE Protocol taught in later modules
- Hippocrates’ emphasis on observation and documentation is the foundation of evidence-based practice

Slide 7: Historical Timeline

- Use this slide to synthesise the ancient period
- The comparative table helps students see patterns across civilisations
- Source criticism: teach students to evaluate historical claims — what is documented vs. what is claimed?
- Exercise: Give students a false historical claim and ask them to evaluate it

Common Student Questions

Q: Which civilisation “invented” cupping? A: We don’t know. Cupping likely emerged independently in multiple civilisations. The earliest WRITTEN record is Egyptian (c. 1550 BCE), but archaeological evidence may predate this. Claims about “invention” are usually nationalistic, not scholarly.

Q: Is TCM cupping “the same” as Hijama? A: The physical technique of creating suction on the skin is similar, but the theoretical frameworks differ significantly. TCM explains cupping through meridians and qi; Hijama is grounded in the Sunnah and Islamic scholarly tradition. Both produce therapeutic effects.

Q: Was Hippocrates Muslim? A: No — Hippocrates lived centuries before Islam. However, Islamic scholars respected and built upon Greek medical knowledge through the translation movement. This synthesis is part of the Islamic scholarly tradition.

Lesson 2.2 — The Islamic Golden Age to Today (4 hours)

Timing Breakdown

Segment	Duration	Activity
Opening	10 min	Recap ancient civilisations, niyyah
Islamic Golden Age	30 min	Slide 9: Translation movement, bimaristans, Tibb al-Nabawi
Key Scholars	45 min	Slide 10: Four major scholars and their contributions
Ibn al-Qayyim Deep Dive	30 min	Slide 11: Zad al-Ma’ ad, methodology, lasting significance
Break	15 min	
Decline and Revival	30 min	Slide 12: Colonial impact, modern revival, global spread
Contemporary Landscape	30 min	Slide 13: Evidence milestones, current challenges
Knowledge Check	15 min	Quiz and discussion
Practical Activity	20 min	Scholarly methodology exercise
Reflection	10 min	“Responsibility of revival” prompt
Summary	5 min	Module summary, preview Module 3

Teaching Notes by Slide

Slide 9: The Islamic Golden Age

- This is the heart of the module for SRA students
- Three transformative factors: translation movement, bimaristan system, Tibb al-Nabawi
- Emphasise that Islamic scholars didn’t just preserve knowledge — they advanced it
- The bimaristan was the first true hospital system in history — not just treatment, but teaching and research

Slide 10: Key Islamic Scholars

- Present the four scholars as a scholarly lineage, not isolated individuals
- Each built upon the work of predecessors
- Their shared methodology: source verification, synthesis, original observation, systematic organisation
- Connect to SRA: “This is the tradition we continue”

Slide 11: Ibn al-Qayyim Deep Dive

- This is the most important slide in the module for SRA practitioners
- Read the quote from Zad al-Ma’ad aloud
- Explain his three-fold methodology: source verification, clinical observation, systematic organisation
- Emphasise: Zad al-Ma’ad is not just a religious text — it is a clinical manual
- Students should understand why this text remains the foundation of Hijama practice

Slide 12: Decline and Revival

- Be sensitive: colonialism is a painful topic for many students
- Frame it as historical fact, not political polemic
- The decline was real and documented — loss of manuscripts, closure of institutions
- The revival is equally real and ongoing
- Global spread map: ask students to share their regional experiences

Slide 13: Contemporary Landscape

- Research milestones: 2012 → 2022 → 2024 → 2026
- Emphasise the trajectory: evidence is growing and strengthening
- Current challenges: standardisation, research quality, regulation, integration
- SRA’s role: addressing all four challenges through the Academy Framework
- This slide bridges to Module 3 (Hijama in Islam) and Module 4 (Evidence-Based Medicine)

Common Student Questions

Q: Why did traditional medicine decline under colonialism? A: Colonial powers systematically replaced indigenous institutions with Western systems. Bimaristans and madrasas were closed. Manuscripts were lost or removed to European collections. Traditional practitioners lost legal status. This was not a natural evolution — it was a deliberate policy of cultural replacement.

Q: Is there any value in pre-Islamic cupping traditions? A: Yes — the Prophet (peace be upon him) didn’t reject pre-existing knowledge. He purified and elevated it. Egyptian, Greek, and Chinese observations about cupping’s effects are clinically relevant, even if their theoretical frameworks differ from Islam.

Q: How do I respond when someone says Hijama is “just traditional” with no evidence? A: Direct them to the 177 RCTs in the 2022 systematic review. Hijama has both a long tradition AND a growing evidence base. These are complementary, not contradictory.

Assessment Guidance

Formative Assessment (During Module)

Knowledge Check Questions (Slide 14):

1. b) The Ebers Papyrus
2. b) Combined authentic Hadith with clinical observation
3. a) Blood, phlegm, black bile, yellow bile
4. c) The translation movement

Case Study 1 — The Skeptical Colleague:

- Look for: respectful but confident defence, citation of evidence, professionalism
- Common error: Students may become defensive or make claims beyond the evidence

Case Study 2 — The Traditionalist Patient:

- Look for: respect for tradition balanced with evidence-informed practice, clear communication
- Common error: Dismissing the grandmother's views or failing to explain evidence gently

Summative Assessment (Module 2 Content in Final Exam)

Module 2 content will appear in:

- **Written Examination:** MCQs on historical facts, scholar contributions, timeline
- **OSCE Station 2 (Consent and Communication):** May include questions about Hijama's history
- **Portfolio:** Students may reference historical context in reflective commentaries

Practical Activities

Activity 1: Timeline Construction (Lesson 2.1)

Materials: Timeline handout, sticky notes, markers

Instructions:

1. Give students a blank timeline from 2000 BCE to 2026 CE
2. In groups, students place key events on the timeline
3. Groups compare and correct
4. Discuss: What patterns do you see? What surprised you?

Learning Outcome: Students can place key historical events in chronological order

Activity 2: Scholarly Methodology Exercise (Lesson 2.2)

Materials: Handout with a weak Hadith about Hijama, a clinical claim

Instructions:

1. Students apply Ibn al-Qayyim's methodology:
 - Step 1: Verify the source (is the Hadith authentic?)
 - Step 2: Check existing knowledge (what do scholars say?)
 - Step 3: Add clinical observation (does it match your experience?)
 - Step 4: Organise systematically (how would you document this?)
2. Groups present their analysis
3. Discuss: Why is this methodology important for patient safety?

Learning Outcome: Students can apply the scholarly methodology to evaluate claims

Differentiation Strategies

For Students Without History Background

- Provide a glossary of historical terms (BCE, CE, colonialism, etc.)
- Use the timeline visual aid extensively
- Connect historical facts to clinical relevance

For Students From Regions with Strong Hijama Tradition

- Ask them to share family or community traditions (with respect)
- Challenge them to evaluate their traditions against scholarly sources
- Encourage them to be ambassadors for evidence-informed practice

For Students Who Struggle with Names/Dates

- Focus on concepts rather than memorisation
 - Use the comparative table to show patterns
 - Emphasise understanding over recall
-

Islamic Integration Notes

Key Islamic References in This Module

Reference	Location	Teaching Approach
Prophet's practice of Hijama	Slides 9–11	Connect to authentic Hadith in Module 3
Tibb al-Nabawi tradition	Slide 9	Explain as scholarly discipline, not folk practice

Table 4 – continued

Reference	Location	Teaching Approach
Ibn al-Qayyim’ s methodology	Slide 11	Emphasise source verification as Islamic duty
Qur’ an 3:48	Final slide	Allah teaches wisdom through revelation and reason

Sensitivity Notes

- Colonial history may be emotionally charged — maintain scholarly neutrality
- Respect all students’ cultural backgrounds
- Avoid nationalistic framing — present history as shared human heritage
- Emphasise that Islamic scholarship built upon and surpassed pre-Islamic knowledge

Troubleshooting

Issue	Solution
Student dismisses non-Islamic traditions	Remind them that the Prophet (peace be upon him) valued beneficial knowledge wherever it came from
Student rejects evidence-based approach	Reference Ibn al-Qayyim’ s methodology —he verified claims through observation
Student expresses anger about colonial history	Acknowledge the pain; redirect to the positive story of revival
Technical issue with manuscript images	Have high-resolution images available offline

Post-Module Checklist

- All 15 slides presented
- Both case studies discussed
- All knowledge check questions answered
- Practical activities completed
- Reflection prompts addressed
- Student questions documented
- Attendance recorded
- Any concerns about individual students noted
- Feedback collected

Contact

Academic Director: academic.director@sunnahremedies.academy **Technical Support:** support@sunnahremedies.academy **Emergency (student welfare):** welfare@sunnahremedies.academy

Sunnah Remedies Academy — Institute of Prophetic Medicine

This document is confidential and for SRA faculty use only.